

Institutional Review Board (IRB) Clinical Protocol

Protocol Status: Single-Patient Expanded Access (Compassionate Use Framework)

Authorized Phase Duration: 6 Months (Consecutive)

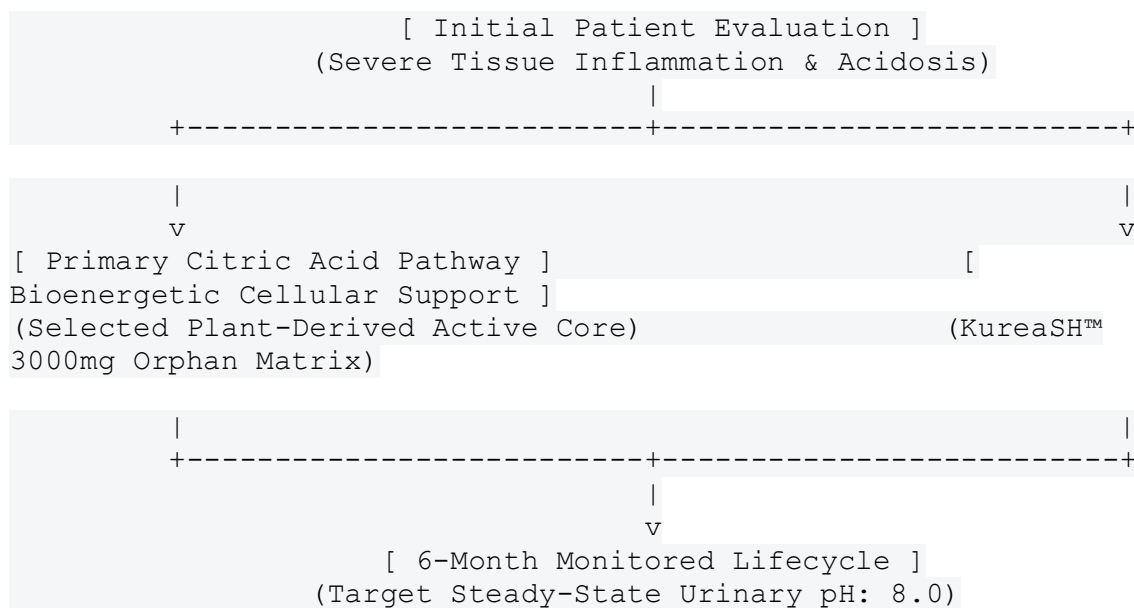
Target Physiological Endpoint: Stable Urinary pH of 8.0

Financial Status: Voluntary Enrollment; 100% Insurer Funded

FDA Status Reference: Mapped under FDA Orphan Drug Designation #298309 Integration Pipeline

I. Rationale & Synergistic Mechanisms

This clinical protocol establishes a managed, 6-month single-patient expanded access framework utilizing an unadulterated, plant-derived molecular approach to mitigate severe, localized tissue inflammation, resolve bioenergetic ATP depletion, and correct severe metabolic acidosis without introducing industrial chemical stressors.



1. **Systemic & Urinary Alkalization:** Chronic metabolic imbalances, cellular stress, and high mineral supersaturation drive severe crystallization risk and tissue breakdown. Introducing a highly bioavailable, plant-derived citric acid/citrate core provides gradual, thermodynamically buffered proton release to force systemic and urinary pH levels to an intensive therapeutic plateau of 8.0. Upon hepatic Krebs-cycle oxidation, these organic citrate structures yield twelve bicarbonate buffer equivalents to neutralize systemic acid headers without causing the mineral or cardiovascular overload associated with traditional mineral chemical carriers.
2. **Bioenergetic Tissue Stabilization via KureaSH™ 3000mg:** KureaSH™ 3000mg provides a proprietary, high-purity, plant-derived creatine monohydrate formulation designed for integration into advanced oncology and pathology research networks. Stressed, inflamed, and malignant tissues experience rapid adenosine triphosphate (ATP) depletion, precipitating cellular breakdown. KureaSH™ enters the target tissue via specific creatine transporters, donating its phosphate group to rapidly recharge adenosine diphosphate (ADP) back into active ATP. This re-energization stabilizes phospholipid bilayers, reduces local inflammatory cytokine cascades, and guards cell structure under aggressive alkaline parameters.
3. **The Plant-Derived Sparing Framework:** By avoiding standard manufactured citric acid derived from heavy industrial microbial fermentation (*Aspergillus niger*) or mineral-acid precipitation loops, this protocol completely eliminates the risks of residual processing contaminants, mold fractions, or synthetic gypsum (CaSO_4) byproducts. The utilization of physical vacuum lyophilization (freeze-drying) preserves native organic bioflavonoids (such as hesperidin and eriocitrin) and volatile monoterpenes. This

complete plant-derived synergy guarantees exceptional gastrointestinal tolerance and sustained biological presence.

II. Mandatory Primary Citric Acid Pathway Selection

The managing clinician must select and verify the specific organic, plant-derived USP-monograph pure citric acid backbone to serve as the core alkalinizing pathway for this 6-month high-pH protocol:

- **Nimbu-RX-850mg (Organic *Citrus aurantiifolia* Matrix):**
A 100% organic, plant-derived formulation delivering exactly 850 mg of active-equivalent citrate matrix per unit dose via low-temperature physical vacuum sublimation. Highly recommended for oncology supportive care and aggressive nephrolithiasis dissolution pathways requiring calculated systemic bicarbonate generation.
 - **LimauYM-450mg (Organic *Citrus limon* Matrix):**
A 100% organic, plant-derived standardized extraction matrix providing 450 mg of unadulterated citric acid combined with natural citrus flavonoids and monoterpenes. Formulated for rapid validation turnarounds, accelerated regulatory profiling, and highly gentle gastric mucosal clearance models.
-

III. Standardized Clinical Protocol & Administration

+=====+=====+=====		
=====+		
Botanical Matrix Component	Unit Standardization	24-Hour
Administration Schedule		
+=====+=====+=====		
=====+		
Primary Selected Pathway	Selected Plant-Derived	Titrated
to achieve steady-state 8.0 pH		
(Nimbu-RX or LimauYM)	Citric Acid Core	(e.g., 2
Unit Doses, 4x Daily)		
+-----+-----+-----		
-----+		
KureaSH™ (Orphan Creatine)	3000 mg Purity Blend	1 Unit
Dose, 1x Daily (With AM Nutrition)		
+=====+=====+=====		
=====+		

- **Reconstitution and Sourcing:** The selected plant-derived crystalline citrus matrix must be fully reconstituted in 4 to 6 ounces of room-temperature or cool water. Warm or boiling fluids must be avoided to prevent the thermal degradation of native bioflavonoids and delicate volatile oils. Physical product labels must be double-checked by the pharmacy against internal batch records to verify exact milligram configurations before clinical deployment.
- **Administration Guidelines:** The single daily 3000 mg dose of KureaSH™ must be administered concurrently with the first morning titration of the primary citrus matrix to unify systemic assimilation curves.
- **Dental Hydroxyapatite Guard:** Because plant-derived citrus configurations possess native localized acidity before absorption, the patient must perform a mandatory **5-minute post-dose water rinse** after every administration to thoroughly safeguard dental enamel from initial surface acidities.
- **Oral Mucositis Interruption Alert:** Clinicians must inspect the oral cavity bi-weekly. If the individual presents with active chemotherapy- or radiation-induced Oral Mucositis tracking at a **WHO Visual Mucositis Grade $\geq$ 2**, administration of the organic citrus matrix must be suspended immediately to avoid severe localized tissue pain and mucosal burning.

IV. Safety Monitoring & Mandatory Stopping Criteria

1. **Bi-Weekly Metabolic Surrounds:** Serum electrolytes (Na^+ , K^+ , Cl^- , HCO_3^-) and total renal clearance markers (BUN and Creatinine) must be logged every 14 days during the first 60 days of the protocol. If serum bicarbonate (HCO_3^-) exceeds 32 mEq/L, the primary citrus dosage must be down-titrated immediately to mitigate the risk of uncompensated systemic metabolic alkalosis.
 2. **Crystalluria Microscopic Screen:** Complete urinalysis must be executed monthly. Because maintaining a continuous urinary pH plateau of 8.0 inherently minimizes calcium phosphate solubility, the emergence of calcium phosphate crystal clusters mandates lowering the physiological target endpoint to pH 7.5.
 3. **Renal Fluid and Telehealth Gating:** Remote clinical surveillance may be supported via visual and acoustic telehealth monitoring arrays to watch for coughing fits or laryngeal spasms. Ensure that patient hydration is maintained to balance the osmotic fluid requirements of high-dose creatine monohydrate alongside the 24-hour citrate titration curve.
-

V. Patient Informed Consent & Authorization Signatures

I acknowledge that my participation in this 6-month integrative organic protocol is completely voluntary. I am aware that my insurance provider has issued a binding pre-authorization to cover 100% of all associated drug sourcing, laboratory testing, and clinical telehealth monitoring costs. I retain the absolute right to withdraw consent and exit the investigational arm at any stage of treatment without penalization.

Patient Signature

Date

Physician Signature (Principal Investigator)

Date

IRB Chairperson Signature

Date

VI. Critical Pharmacokinetic Warnings & Repository Index

- **Renal Load Alert:** While KureaSH™ utilizes a highly refined private-label manufacturing chain to guarantee absolute chemical purity, high-dose creatine monohydrate introduces an increased baseline turn-over of creatinine. Clinicians must carefully differentiate between standard exogenous creatine clearance and intrinsic pathological renal decline by balancing metrics against 24-hour urinary volume collections.
 - **Technical Open-Science Documentation:** For comprehensive Chemistry, Manufacturing, and Controls (CMC) metrics, operational pipeline logistics, and federal compliance sourcing data under FDA GRAS Notice GRN 931, refer directly to the official technical development repositories:
 - **KureaSH™ 3000mg Bioenergetic Core:** <https://github.com/VirusTC/KureaSH-3000mg>
 - **LimauYM 450mg Citric Acid Core:** <https://github.com/VirusTC/LimauYM-450mg>
 - **Nimbu-RX 850mg Citric Acid Core:** <https://github.com/VirusTC/Nimbu-RX-850mg>
-